

NC Medicaid

April 2026 PDL Impact Analysis

Off-Cycle Preferred Drug List Changes Effective April 1, 2026

Report Date: April 02, 2026

Data Sources: CMS State Drug Utilization (CY2024, H1-2025)

NADAC (National Average Drug Acquisition Cost) as of 04/01/2026

NC DHHS PDL Change Notification 04/02/2026

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GROSS REIMBURSEMENT ANALYSIS (PRE-REBATE)

1. Executive Summary

On April 2, 2026, NC DHHS issued off-cycle Preferred Drug List changes effective April 1, 2026, citing 'significant fiscal impact.' The most consequential change: Humira (adalimumab) moved from co-preferred to NON-PREFERRED status across all dosage forms, representing a significant escalation from the January 2026 PDL approach.

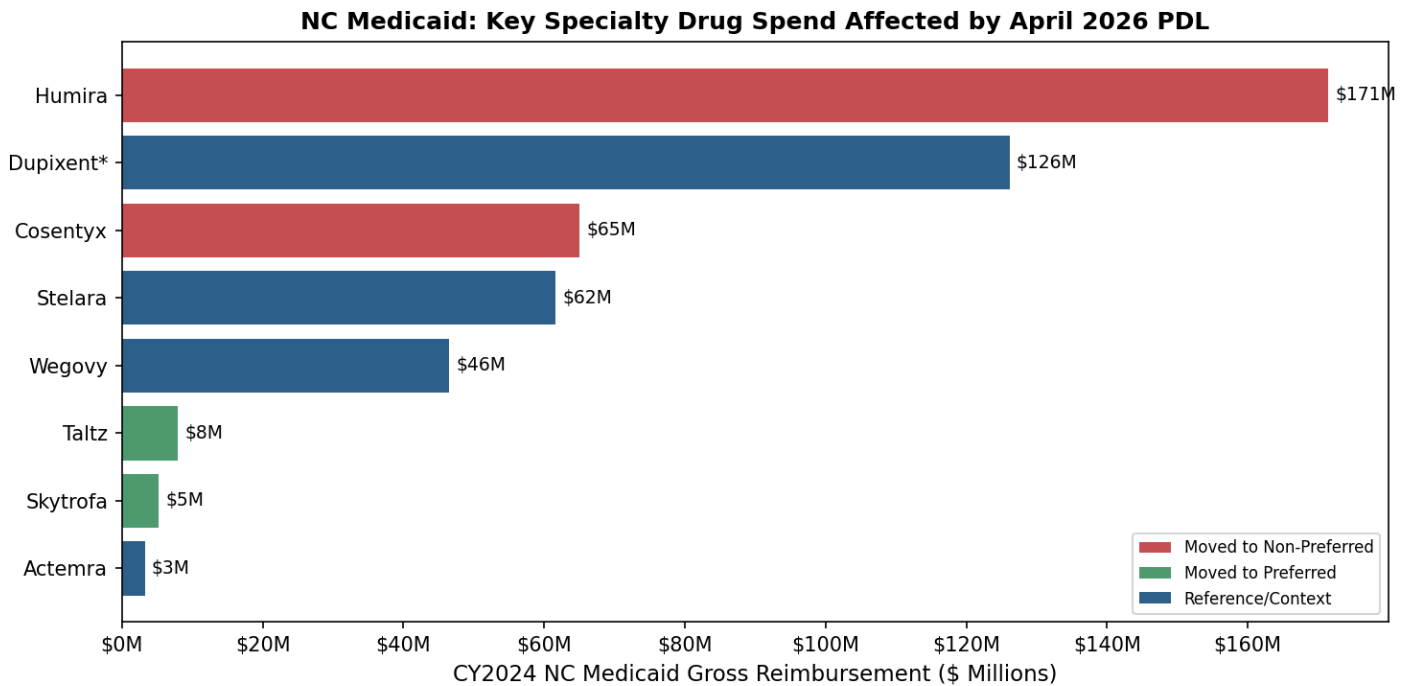
Additional high-impact changes include Cosentyx moved to non-preferred (replaced by Taltz), Starjemza promoted as preferred ustekinumab biosimilar, Tyenne promoted as preferred tocilizumab biosimilar, and Wegovy oral tablet added as preferred.

Key Financial Metrics

Total CY2024 Spend on Affected Drugs:	\$301,280,167
Annualized H1-2025 Wegovy Spend:	\$313,509,378
Humira 2-Year Gross Savings (Moderate):	\$74,050,422
Humira 2-Year State Savings (Moderate):	\$26,508,705
SFY26 State Share (non-expansion):	35.38% (FMAP 0.6462)
SFY27 State Share (non-expansion):	35.84% (FMAP 0.6416)

IMPORTANT: This analysis uses GROSS reimbursement only (pre-rebate). Humira carries substantial MDRP (Medicaid Drug Rebate Program) rebates estimated at ~23.1%, while biosimilars have lower rebates (~13%) and minimal CPI penalties. The Cosentyx-to-Taltz and Stelara-to-Starjemza swaps are believed to be primarily rebate-driven. Complete net fiscal analysis requires confidential supplemental rebate data.

2. Affected Drug Spend Overview



The chart above shows CY2024 NC Medicaid gross reimbursement for the major specialty drugs affected by the April 2026 PDL change. Red bars indicate drugs moved to non-preferred status; green bars indicate drugs promoted to preferred. Blue bars are reference drugs providing context.

* Dupixent is shown for context as the atopic dermatitis market leader. Ebglyss (lebrikizumab), newly preferred for atopic dermatitis, is actually more expensive per unit than Dupixent by NADAC (\$1,710 vs \$1,010), suggesting the preferred status is rebate-negotiated.

3. PDL Change Summary

Moved to Non-Preferred

Drug	Category	Previous Status	CY2024 Spend
Humira (all forms)	TNF-alpha	Co-Preferred	\$171,393,345
Adalimumab-adaz	TNF-alpha Biosimilar	Preferred	Minimal
Adalimumab-adbm	TNF-alpha Biosimilar	Preferred	Minimal
Cosentyx	IL-17 Inhibitor	Preferred	\$65,055,253
Steqeyma	IL-12/23 Biosimilar	Preferred	New Product

Moved to Preferred

Drug	Category	Replaces	NADAC/Unit
Taltz (ixekizumab)	IL-17 Inhibitor	Cosentyx	\$7,020.13
Starjemza	IL-12/23 Biosimilar	Stelara	No NADAC Yet
Tyenne	IL-6 Biosimilar	Actemra	\$819.74
Skytrofa	Growth Hormone	Various	No NADAC
Ebglyss	IL-13 (Atopic Derm)	Dupixent	\$1,795.81
Wegovy Tablet	GLP-1 Weight Mgmt	Injectable Wegovy	\$43.22
Vtama Cream	Topical Psoriasis	Various	\$24.12

4. Humira Deep Dive: Co-Preferred to Non-Preferred

The January 2026 PDL made adalimumab biosimilars co-preferred with Humira. The April 2026 change escalates this to making Humira fully NON-PREFERRED, which is a significantly stronger signal. Non-preferred status typically requires prior authorization, creating a direct mechanism to drive biosimilar adoption.

However, as of April 2, 2026, NC DHB has NOT updated Humira PA criteria to require biosimilar trial/failure first (unlike Stelara, which has updated criteria). This is a critical gap identified by Standard Plan pharmacy directors.

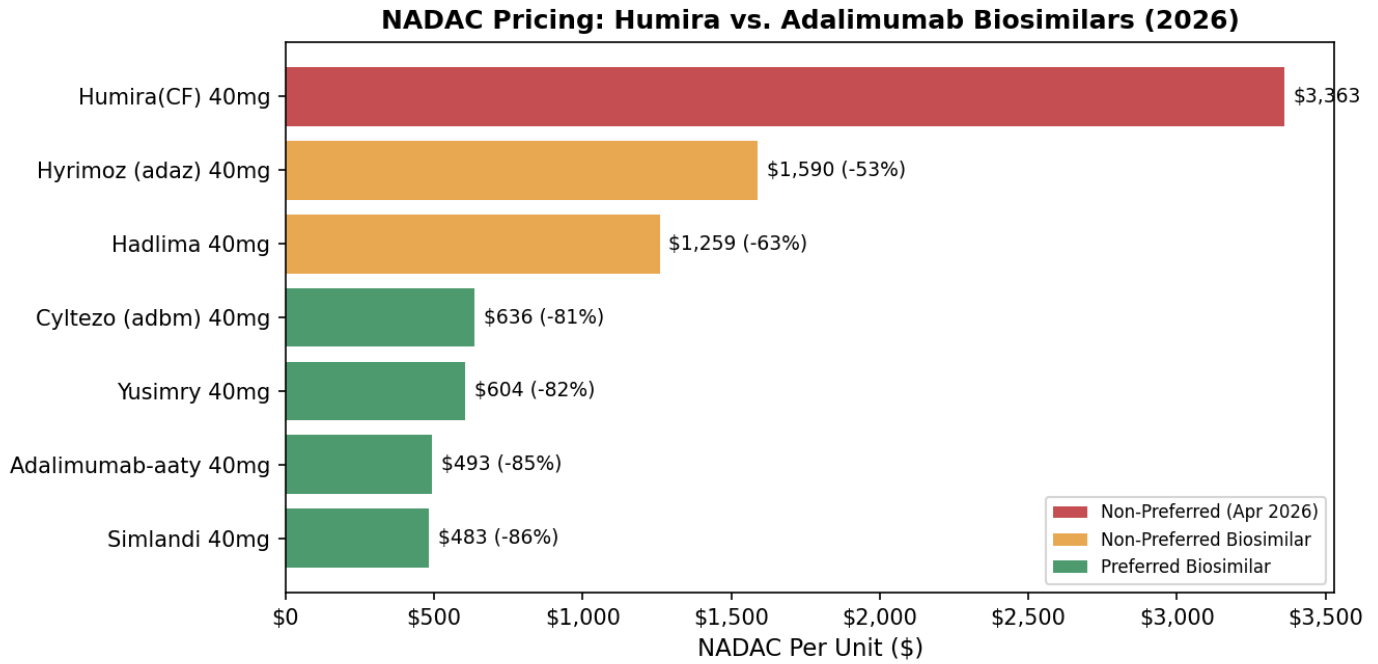
CY2024 Baseline Utilization

Total Units Reimbursed:	44,310
Total Prescriptions:	16,238
Gross Total Reimbursed:	\$171,393,345
Medicaid Amount:	\$167,145,049
Cost Per Unit (Actual):	\$3,868.03
NADAC Per Unit (Humira 40mg):	\$3,362.68
Current Biosimilar Adoption:	<1% (NC Medicaid)

H1-2025 Trend

H1-2025 Units:	23,724
H1-2025 Total:	\$91,835,856
YoY Growth (annualized):	7.2%

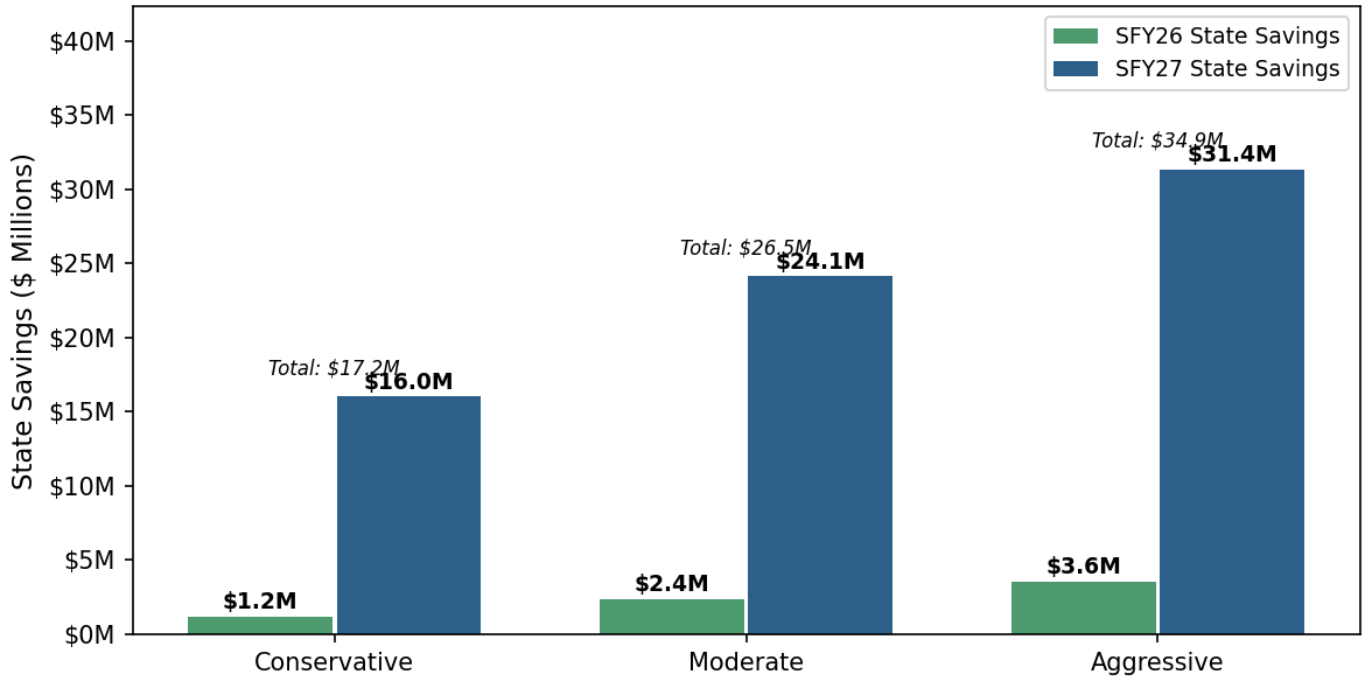
NADAC Pricing: Humira vs. Biosimilars



The cheapest biosimilar (Simlandi) offers an 86% NADAC discount vs. Humira. The blended average across available biosimilars is approximately 73% discount. Note: NADAC reflects acquisition cost only; actual reimbursement includes dispensing fees and varies by plan contract.

Adoption Scenarios & State Savings

**Humira Biosimilar: Projected State Savings by Scenario
(Non-Preferred effective Apr 2026, FMAP-adjusted)**



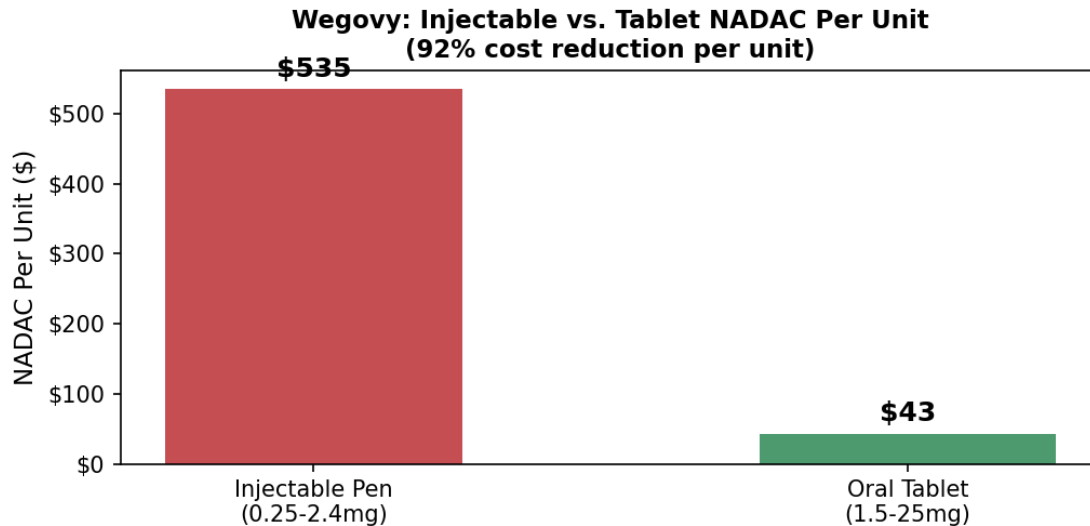
Scenario	SFY26 Gross	SFY27 Gross	SFY26 State	SFY27 State	2-Year State
Conservative	\$3.4M	\$44.8M	\$1.2M	\$16.0M	\$17.2M
Moderate	\$6.7M	\$67.3M	\$2.4M	\$24.1M	\$26.5M
Aggressive	\$10.1M	\$87.5M	\$3.6M	\$31.4M	\$34.9M

State savings use FMAP-derived non-expansion state share: SFY26 = 35.38%, SFY27 = 35.84%. Expansion population (10% state share) would reduce blended state share. SFY26 impact is limited to Q4 only (Apr-Jun 2026) due to April 1 effective date. Non-preferred status accelerates adoption curves compared to prior co-preferred analysis (Jan 2026 PDL).

5. Wegovy: Injectable to Tablet Transition

Wegovy (semaglutide) for weight management represents the fastest-growing drug category in NC Medicaid. CY2024 spend was \$46.5M; H1-2025 annualized is \$313,509,378 -- representing 575% year-over-year growth.

The April 2026 PDL adds Wegovy oral tablet as preferred. The tablet's NADAC is approximately \$43/unit vs. \$434-651/unit for injectable pens -- a 92-93% reduction per unit. However, dosing equivalence and patient compliance may differ between oral and injectable formulations.



Potential Savings (NADAC-Based)

Conversion Rate	Annual Gross Savings
25%	\$10,851,541
50%	\$21,703,081
75%	\$32,554,622

CRITICAL CAVEAT: These savings estimates assume unit-for-unit substitution. Oral semaglutide has different bioavailability and dosing requirements. The actual savings depend on equivalent therapeutic dosing, which may require different unit quantities. The \$43/unit tablet pricing reflects newly published NADAC as of March 18, 2026.

6. Other Therapeutic Category Impacts

Cosentyx -> Taltz (IL-17 Inhibitor)

CY2024 Cosentyx NC Medicaid spend: \$65,055,253. Taltz is 7-8% cheaper per NADAC for comparable dosing (150mg maintenance: Cosentyx \$7,209 vs Taltz \$6,682). However, this swap is likely driven primarily by supplemental rebate negotiations rather than NADAC savings alone. The NADAC-based savings are modest; the real fiscal impact depends on confidential rebate terms.

Stelara -> Starjemza (IL-12/23 Biosimilar)

CY2024 Stelara NC Medicaid spend: \$61,607,303. Starjemza (ustekinumab-aekn) is a biosimilar to Stelara. No NADAC pricing is available yet (too new for CMS NADAC survey). Typical specialty biosimilar discounts range 30-50% initially. At 40% discount with 50% conversion, the potential gross savings would be approximately \$12,321,461/year.

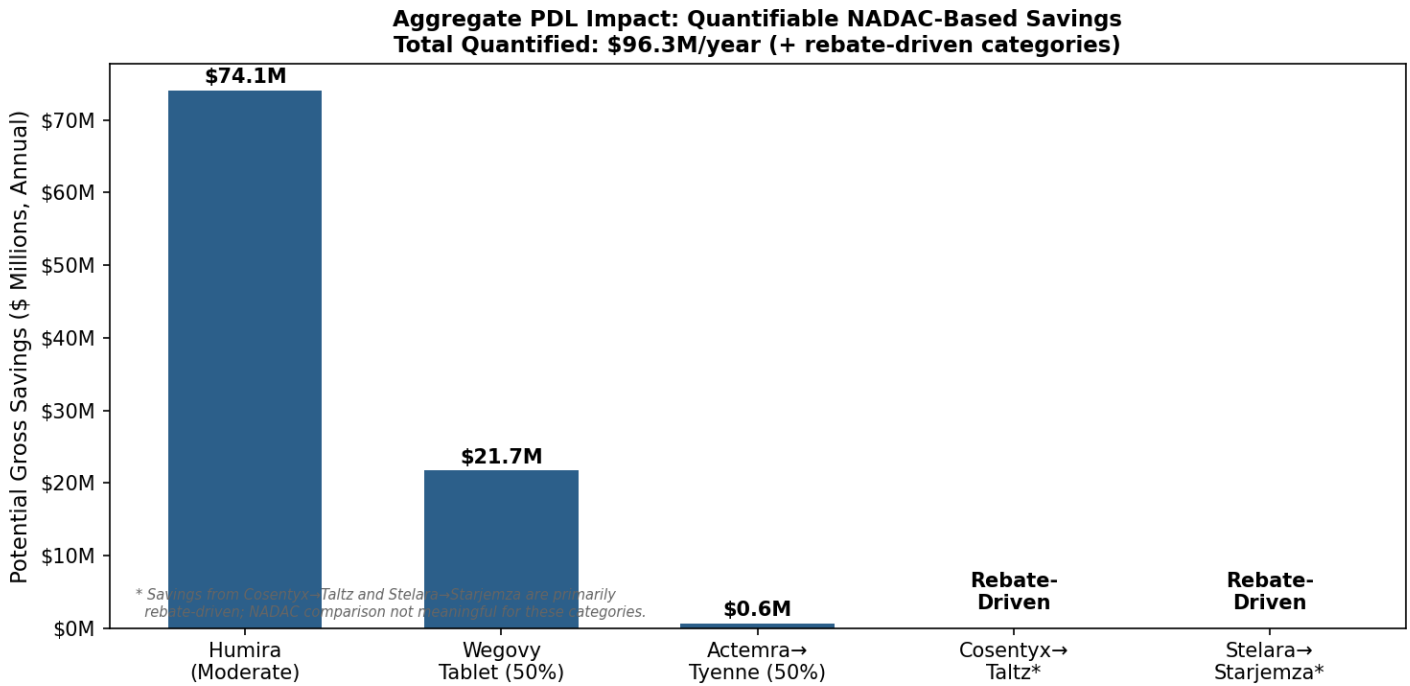
Actemra -> Tyenne (IL-6 Biosimilar)

CY2024 Actemra NC Medicaid spend: \$3,224,266. Tyenne offers a 36% NADAC discount (\$820 vs \$1,273/unit). At 50% conversion: \$578,921.55/year savings. Smaller category but straightforward biosimilar substitution.

Ebglyss (Atopic Dermatitis)

Ebglyss (lebrikizumab) promoted to preferred for atopic dermatitis. NADAC is \$1,710/unit vs Dupixent at \$917-1,615/unit. Ebglyss is NOT cheaper by NADAC -- this is entirely a rebate-negotiated placement. CY2024 Dupixent NC Medicaid spend: \$126,133,368. Any conversion from Dupixent to Ebglyss would increase NADAC-based costs but presumably decrease net costs after manufacturer rebates.

7. Aggregate Financial Impact



NADAC-quantifiable annual gross savings (at moderate adoption):

- Humira biosimilar (Moderate, SFY27 run-rate): ~\$67.3M
- Wegovy tablet (50% conversion): ~\$21.7M
- Actemra->Tyenne (50%): ~\$0.6M
- Stelara->Starjemza (est. 40% disc, 50% conv.): ~\$12.3M

Rebate-driven (not NADAC-quantifiable):

- Cosentyx->Taltz: CY2024 baseline \$65,055,253
- Dupixent->Ebglyss: CY2024 baseline \$126,133,368

NOTE: Gross savings do not account for Medicaid Drug Rebate Program (MDRP) offsets. Humira's rebate rate (~23.1% + CPI penalties) is substantially higher than biosimilar rebates (~13%). Net savings will be lower than gross estimates.

8. Implementation Risk Assessment

Standard Plan (SP) pharmacy directors raised the following concerns in a letter dated March 31, 2026, regarding the April 1, 2026 PDL changes:

Claims System Lag [High]

PDL effective 04/01, but claims system not updated until 04/03/2026. Two-day window of incorrect adjudication.

Exposure: Estimated 2 days × average daily Humira/Cosentyx/Stelara claims

PA Criteria Gap [Critical]

NC DHB has not updated Humira PA criteria to require biosimilar trial/failure first (as done for Stelara). Plans cannot enforce non-preferred status without updated PA criteria.

Exposure: Without PA enforcement, the non-preferred designation has no mechanism to drive biosimilar adoption. Savings projections assume PA is in place.

Member Notification [High]

PHPs received notification on 04/01 at 2:24 PM, formal notice on 04/02 at 9:30 AM. No time for required member notification on complex medication changes.

Exposure: Potential member access issues and regulatory compliance concerns.

Existing PA Management [Medium]

Request to end-date existing Humira PAs on 03/31/2026 to force conversion. Abrupt PA termination for established specialty drug patients raises clinical transition concerns.

Exposure: Patient disruption; potential for adverse outcomes during transition.

The PA criteria gap is the most critical risk. Without updated PA criteria requiring biosimilar trial/failure for Humira, the non-preferred designation lacks an enforcement mechanism. All savings projections in this report assume PA enforcement will be implemented. If the PA criteria gap persists, actual biosimilar adoption rates could be significantly lower than projected.

9. Key Assumptions & Limitations

Data Currency

SDU 2025 covers Q1-Q2 only (Jan-Jun 2025). CY2024 used as full-year baseline. NADAC pricing as of 04/01/2026.

FMAP Rates

Non-expansion: SFY26 FMAP=0.6462, SFY27 FMAP=0.6416. Expansion FMAP=0.90. Analysis uses non-expansion state share as conservative estimate.

Biosimilar Adoption

Assumes PA enforcement is implemented for Humira. Conservative/Moderate/Aggressive scenarios based on other states' experience with non-preferred biologic conversions (OR, WA, NY).

Gross vs Net

ALL estimates are gross reimbursement (pre-rebate). Humira MDRP base rebate is 23.1% plus substantial CPI penalties. Biosimilars have ~13% rebate with minimal CPI penalties. Net savings will be materially lower.

Wegovy Tablet

Unit-for-unit pricing comparison. Oral vs injectable bioavailability differences may require different unit volumes for therapeutic equivalence.

Starjemza/Steqeyma

No NADAC pricing available for Starjemza (too new). Estimated 30-50% biosimilar discount based on specialty drug market norms.

Rebate-Driven Swaps

Cosentyx->Taltz and Dupixent->Ebglyss swaps likely driven by supplemental rebate negotiations. NADAC-based comparison not meaningful.

PDL Effective Date

April 1, 2026. SFY26 Q4 (Apr-Jun 2026) is the only impacted quarter in SFY26. Full-year impact begins SFY27.